CMS Overspending on Prosthetic Services

Executive Summary

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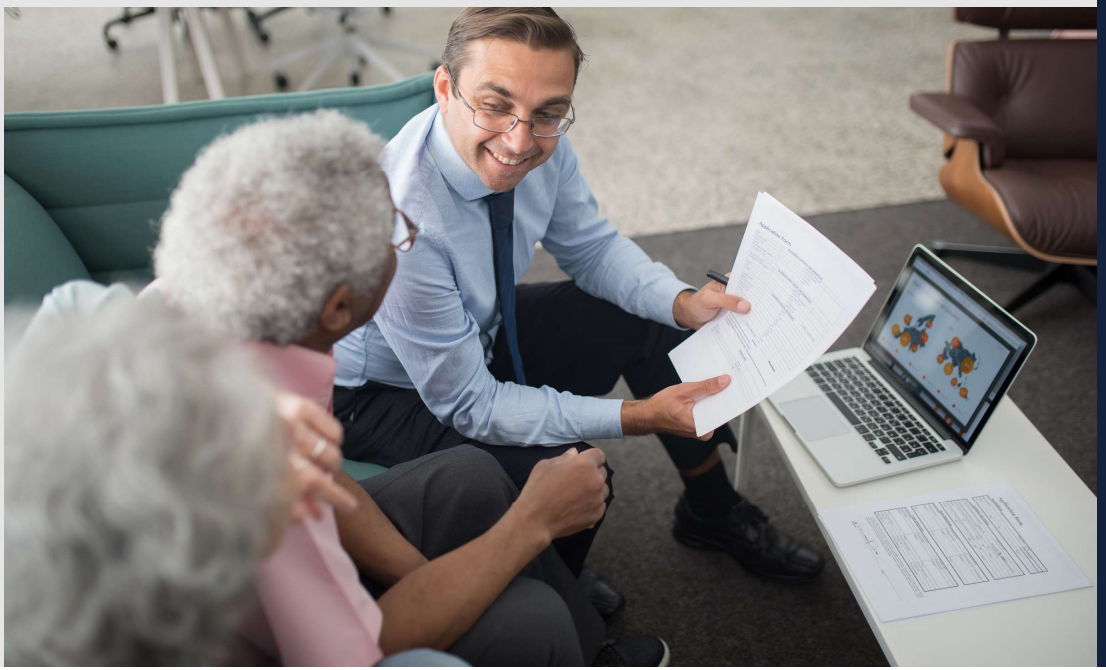

Data

- CMS DMEPOS by Geography & Service
- CMS National Health Expenditure (State Residence)
- Aligned to 2014–2020

Objectives and Goals

1. ID state-level prx spending > average
2. ID most frequently used prx services and find overspending
3. ID most expensive prx services and find overspending
4. Find trends in spending (e.g. by service, by geography, etc.)
5. Forecast future cost demands on CMS based on the data

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Method

- Robust MAD-based z-scores; observations with robust z-score > 2.5 flagged as overspending
- Frequency and total-spend analyses by HCPCS code
- Predictive analysis using linear regression model

Key Findings

- Spending increased from 2014–2019, then dropped sharply in 2020
- Overspending was concentrated in a small number of state–year outliers (largest spike: Georgia 2018).
- Top services: most-used codes included L5673, L5620, L5685, L5679, and L5637; highest total spend included L5673, L5301, L5856, L5700, and L5987.
- Forecast: linear regression used to project 2021–2023 spending; performance was mixed, indicating substantial uncertainty.